Mp10 Patients, Billing and Payments — User's Guide

From the patient at the front desk to the payment on the claim

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1 About this guide

This guide follows a visit through Mp10 from start to finish. The patient is registered, their insurance card is scanned, they sign, an encounter is opened and what they came for is ordered. The biller turns that encounter into a claim, codes it and sends it. The payer answers. The answer is posted, and whatever is still owed is worked until it is paid.

It is written for the people who do that work every day:

- **Front desk and admissions staff** — Part I, *Patients and encounters*.
- **Billers and coders** — Part II, *Claims, coding and collections*.
- **Payment posters** — Part III, *Remittances and payments*.

Each part can be read on its own, but each one hands over to the next, and the reasons behind many of the steps only make sense further along. The insurance card you mark as primary at the front desk is the one the claim bills. The order you enter on the encounter is the line the biller starts from. The remittance you post whole teaches the system which codes a payer refuses. It is worth reading the parts you do not do yourself, at least once.

1.1 Which program you use

Mp10 comes in two editions, and the screens differ a little between them.

Your practice	Patients and encounters	Claims and payments
Professional (physician practice, clinic, imaging centre)	The billing program **PrBill10**, or the appointment calendar **MpXtrmCal10**	**PrBill10**
Institutional (hospital, facility)	**Patients10**	**MpBill10**

This guide describes the professional edition first. Where the institutional edition works differently, a short note headed **Institutional** says how.

Everything here is about the Windows programs. Mp10 Web, the browser version, has its own guide, the *Mp10 Web — User Guide*.

1.2 Other guides this one points to

Several subjects already have a guide of their own. This one tells you when you need them rather than repeating them:

Guide	What it covers
Mp10 Orders — User's Guide	Setting up the order catalogue, order labels, HL7 orders and results
Mp10 Dashboard — User's Guide	The dashboard cards, the full list of claim checks and their tags, unbilled encounters
Mp10 Web — Signature Pads and *Mp10 — Signature Helper*	Installing the signature pad, and the text and options it shows the patient
Mp10 AutoTasks — Administrator's Guide	Downloading claim responses and remittances, and reading remittance PDFs automatically

1.3 Who can do what

What you can do depends on the groups your user account belongs to. Your administrator sets this up.

Group	What it opens
Admission	Patients and encounters. In PrBill10 it also shows the *Encounters Table* button on the toolbar
BillingUsers	Creating and opening claims, from encounters and from the patient record
Supervisors	Deleting an insurance card, changing an encounter's date after it is saved, moving charges between encounters, adjusting balances in bulk, collecting clearinghouse files by hand
Administrators	The same billing work as BillingUsers, deleting an unposted remittance, adjusting balances in bulk, collecting clearinghouse files by hand
AdjustmentAuthUsers	Authorizing adjustments. On its own it opens nothing

If a button is missing for you, or a screen says **Check user permissions**, your account is not in the group that screen needs. That is usually correct; ask your administrator if you think it is not.

1.4 How the screens behave

A few habits are the same on every grid in Mp10:

- **Click a column header to sort by it.** The active header turns red.
- **Type while a grid has focus to jump** to the first row that starts with what you typed, in the column the grid is sorted by. What you have typed shows in the *Incremental Search* box on the left.
- **F8 (or Ctrl+F) opens the Search Dialog.** It jumps to the first match in the sorted column. It does not hide the other rows.
- **The Filter panel on the left hides rows.** It works only while the filter is switched on: click the LED or the word *Filter* to turn it on and off.
- **Enter or F7 opens** the highlighted row. **Insert or F5 adds** a new one.
- **Right-click a grid** for printing, *Excel*, *Change Font*, and *Save Current Grid State*, which keeps your column widths and order for next time.

Part I — Patients and encounters

Finding and registering the patient, their ID, signature and insurance cards, opening the encounter, ordering what they came for, and seeing which visits are still waiting to be billed.

2 Finding a patient

Open the patients list with the **Patients Table** button on the toolbar (or *Tables* → *Patients*). The *Patients Data Grid* lists every patient, sorted by name.

There are three ways to find someone:

1. **Type the name.** With the grid sorted by *Name*, type the last name followed by the first: `GARCIA MARIA`. The grid jumps as you type. It matches from the start of the name, so you do not need to type all of it.
2. **Search (F8).** Sort by the column you know — *MedRec*, *SSN*, *Name*, *DOB*, *City*, *Insurance* or *Creation* — then press F8 and type the value. For a record number you do not need the leading zeros; the search adds them.
3. **Filter.** In the *Filter* panel on the left, fill in any of *Last*, *Name*, *SSN*, *City*, a range of birth dates (*dob from*, *dob thru*) or a range of creation dates, and switch the filter on. Last, first name and SSN match anywhere in the field, so `ROD` finds RODRIGUEZ and RODNEY alike. The number of patients found shows under the search box.

You cannot search by phone number.

Small icons in the grid tell you more about a patient before you open them:

Icon	Where	Means	Click it to
Paperclip or note	*MedRec*	The patient has attachments or notes	Open the attachments
Tag	*Name*	The patient has tags	—
271 (green or red)	*Insurance*	An eligibility answer is on file, accepted or not	Open the eligibility window

2.1 Possible duplicate patients

Two records for the same person cause trouble for years: the history is split, claims go out under two record numbers, and payments land on the wrong one. Mp10 catches most duplicates when the patient is entered (next chapter), and the dashboard's *Patient records* card lists the ones that slipped through as **possible duplicate patients**: records with the same last name, first name, birth date and city. From that list, highlight the record to keep and press **Merge group…**.

A record number can also be changed or merged by hand with **Chng Record** in the patients grid's side panel. Mp10 merges two records only when the name, date of birth and SSN all match; otherwise it refuses.

3 Adding and editing a patient

3.1 Opening the patient window

- **New patient:** press Insert (or F5), or click **New Patient** in the side panel.
- **Existing patient:** double-click the row, or press Enter.

The patient window has these tabs in PrBill10: **Demographics**, **Insurance**, **Encounters**, **Claims**, **Letters** and **Pre-Auths**. The calendar shows only the first three.

At the bottom are **Save** and **Exit**. Save keeps the window open, so you can carry on with the insurance and the encounter. **Ctrl+S** saves and closes in one step; **Ctrl+E** exits.

> **Institutional:** Patients10 adds an **Address** tab (postal and physical address, employer) and shows the encounters and claims of the patient read-only on their tabs.

3.2 The Demographics tab

Type the patient's details: *Last Name*, *First Name*, *SSN*, *DOB*, address, phones, e-mail, marital information and an emergency contact.

- **Last Name holds both surnames** (`RIVERA LOPEZ`). There is no separate second-surname field.
- Names and the SSN are kept in capitals. Mp10 removes anything that is not a letter, a space or a hyphen from a name when you save.
- **City and Zip** have a search button. Pick either one and the other two (city, state, zip) fill in together.
- Phone numbers start with the practice's area code.
- *Age* is worked out from the date of birth.

Only the last name is required. A future date of birth is refused with *Invalid DOB*, and an e-mail address that does not look like one asks you to confirm.

3.3 The record number

The record number (*MedRec*) is given **when you first save** the patient, not when you open the window. You cannot type it. Once saved, the window's title shows it.

Several things are unavailable until the patient has been saved once, because they need the record number: the insurance cards, the HIPAA signature, the encounters and tags. **Save the patient first, then carry on.**

3.4 When Mp10 thinks the patient already exists

On a new patient, as you leave the name, SSN or date-of-birth fields, Mp10 looks for someone already on file with the same SSN, or a similar name. If it finds one, a window shows that patient's name, SSN, date of birth and address, with two buttons:

- **Jump to Existing Pte** closes the new record and takes you to the one on file. Choose this when it is the same person.
- **Continue W/New Pte** carries on with the new record. Choose this only when you are sure it is someone else — two relatives with similar names, for instance.

Your administrator can turn this check off, but should not.

3.5 Saving and the primary insurance

When you save a patient who has insurance cards, one of them must be marked **Primary**. If none is, Mp10 refuses with *Can't save: no primary insurance* and asks you to open the card that should be primary, tick *Primary*, save it, and save the patient again.

When the patient is saved, the primary card's insurance becomes the *Insurance* shown for the patient in the grid and on reports.

4 The patient's photo ID

The **ID** box on the Demographics tab holds a picture of the patient's driver's licence or ID card. It is how anyone in the practice, now or later, can confirm the person in front of them is the patient on file.

Button	What it does
Scanner (*Capture Image From Scanner*)	Scans the card from the desk scanner
Database (*Import id card image from file*)	Loads a JPG file instead
Magnifier (*Zoom IN*), or click the picture	Shows the picture full size
Delete (*Remove id image*)	Removes the picture
OCR (*Read the names, sex and address off the ID*)	Reads the ID on the screen into the fields

To scan: place the card anywhere on the scanner bed and click the scanner button. Mp10 finds the card on the bed and keeps only the card. **The picture is stored when you save the patient**, so press Save afterwards.

A patient has one ID picture. Scanning again replaces it.

4.1 Reading the ID into the fields

When you scan or import an ID, Mp10 reads it and fills in whatever is still blank: **last names, first name, sex, street, city, state and zip**. Fields it filled turn pale yellow, and the line under the tabs says what was read. Fields that already had something in them are left alone.

The **OCR** button under the picture does the same for an ID that is already on the screen, but it **overwrites** the fields with what the card says. Use it when the patient brings a new licence with a new address: scan it, press OCR, check the fields against the card, and Save. Nothing is stored until you save.

What it reads is a proposal. Always compare the yellow fields with the card: a worn or faded card can turn a letter into another, and the card prints the full given names where the record may hold only the first.

Two things to know:

- Mp10 scans IDs at a higher resolution than insurance cards, because a licence's print is small. The scan takes a little longer. A card that was stored before this version, at the old resolution, may read nothing — Mp10 says so; type the details by hand as before.
- If the OCR button says the card reader is not installed on the workstation, the reader's files have not reached that machine yet. Ask your administrator; the fields can still be typed.

For anything else — the back of the ID, a referral, a signed consent form — use **Attachments** in the patients grid's side panel. There you can scan, import JPG or PDF files, scan several pages into one PDF (*Scan to PDFs*), and give each document a type and a date.

If the scanner does not respond, or the wrong scanner is used, ask your administrator: the scanner each workstation uses is a setting.

5 The HIPAA signature

The patient signs the practice's HIPAA notice once, on the signature pad, and the signature is kept on their record. The **HIPAA Signature** box on the Demographics tab shows it, with the date and time it was taken underneath.

5.1 Taking it

1. Save the patient first. The pen button stays disabled on a new, unsaved patient.
2. Click the pen, **Pickup Patient Signature using signing pad**.
3. The pad shows the practice's notice, a page at a time. The patient taps the arrow on the pad to move to the next page. While they read, your screen shows *Wait for user input* with a Cancel button.
4. The patient signs and taps **OK** on the pad (or **CLEAR** to start again).
5. Your screen shows the signature in the *Signature Dialog*. Click **Accept** to keep it, or **Cancel** to throw it away.

Mp10 stores the signature, the time, who took it, and a fingerprint of the exact text the patient was shown. If the notice is reworded later, you can still tell which wording each patient signed.

Taking a signature again replaces the old one, without asking. Only press the pen when a new signature is wanted.

The printer button beside it, **Print HIPAA documents with patient signature printed**, prints the practice's HIPAA document with the patient's signature on it.

5.2 The text on the pad

What the pad shows is set once by the administrator, in the system registry under **TOPAZ / Patient-Record-DisplayMessage**. It can be plain text, or a more complete setting that also chooses the font, the pad model and a list of options for the patient to tap. The format is described in the *Mp10 Web — Signature Pads* guide, chapter *Using it*, and in the *Mp10 — Signature Helper* guide. Those guides describe the setting for the encounter signature; the HIPAA setting takes exactly the same format.

The dashboard's *Patient records* card counts the patients seen in the last 90 days who have no HIPAA signature, so the ones missed are easy to find.

5.3 If the pad does not respond

Topaz signature device could not be activated or *TOPAZ signature pad is not found or missing required components* means this workstation's pad is not connected or its software is not installed. Check the cable first; the installation is covered in the *Mp10 Web — Signature Pads* guide, chapter *Installing it — IT task*.

6 Insurance cards

The **Insurance** tab of the patient window lists every insurance card the patient has ever presented, newest and oldest together. Each row is one card. Below the list are the card's details, and on the right, the picture of the card.

6.1 Why the card history matters

Patients change insurance. When they do, **do not overwrite the old card with the new one.** Add the new card as a new row, make it primary, and leave the old one on file. The old card is the record of who was billed for the earlier visits: when a payer asks about a claim from two years ago, or a payment arrives late for an old plan, the card that was in force then is still there, with its picture.

6.2 The fields

Field	What to enter
Member Number	The member or subscriber ID, exactly as on the card
Group	The group number. **Fill it in whenever the card shows one** — see below
Insurance Co.	The payer. Use the search button to pick from the payers your practice has set up
Effective Date, *Exp Date*	When the coverage starts and ends
Insured First, Last Names	The subscriber — the person who holds the policy
Relationship	The patient's relationship to the subscriber: Self, Spouse, Child, Other… Choosing *Self* fills in the subscriber's name from the patient
DOB	The subscriber's date of birth, as the payer has it
Name as on card, *Last as on card*	The name printed on the card, if it differs
Coverage Codes, *Ded Amt*	Deductible information
Primary, *Secondary*	Which payer is billed first and second

For Medicare, Mp10 checks that the member number looks like a Medicare Beneficiary Identifier and asks you to confirm if it does not.

The *Group* matters because when Mp10 writes the electronic claim it chooses, among the patient's cards for that payer, the one that has a group number. A new card left without a group can mean the claim goes out with the old card's details.

6.3 Adding a new card

1. Save the patient if it is new — the Insurance tab works only on a saved patient.
2. Click **New**.
3. Fill in the fields.
4. Scan the card (next section).
5. Tick **Primary** if this is now the payer to bill first, or **Secondary**, or neither.
6. Click **Save**.

To change a card, highlight it, click **Modify**, make the change and click **Save**. **Cancel** throws your changes away. Double-clicking a row is the same as Modify.

6.4 Making the newest card primary, and keeping the old one

When you save a card marked **Primary**, Mp10 takes the Primary mark off whichever card had it before. That card stays on file, unmarked, as history, with its picture. The same happens with **Secondary**. There is always at most one primary and one secondary card.

So, when a patient brings a new card:

1. Click **New** and enter the new card.
2. Tick **Primary**.
3. Save.

The old card is now history and the new one is billed. If you know the old coverage ended, open the old card with Modify and enter its *Exp Date*. Mp10 does not set it for you.

What uses which card:

- **A new encounter** takes the primary card's payer.
- **A new claim** takes the primary card's payer and member number, and the secondary card's payer as its complementary insurance.
- **An eligibility check** asks the primary payer.
- **Existing encounters and claims keep the payer they were created with.** Changing the primary card later does not change them.

Only **Supervisors** can delete a card. That is deliberate: a card that is no longer valid should be left as history, not deleted.

6.5 The picture of the card

Each card row keeps one picture of the card. While you are adding or modifying a card:

- **Scan** captures it from the desk scanner. Place the card anywhere on the bed: Mp10 finds it and keeps only the card.
- **Import card image from file** loads a JPG or PDF file.
- **Delete** removes the picture.

The picture is stored when you save the card. To see it later, highlight the card in the list: its picture shows on the right. **Preview**, or a click on the picture, shows it full size.

A card row holds one picture, normally the front. Keep the back of the card, when you need it, in the patient's **Attachments**.

6.6 Reading the card into the fields

When you scan or import a card on a new or modified row, Mp10 reads it and fills in the **member number** and the **group** if they are still blank. Fields it filled turn pale yellow, and the line under the tabs says what was read and which payer the card was recognised as. Typed values are never touched.

The **OCR** button:

- **While you are adding or modifying the card**, it reads the card on the screen and **overwrites** the member number and group with what the card says. Check them against the card, then Save or Cancel.
- **While you are only looking at a card**, it reads the card and reports, on the line under the tabs, what the card says next to what the record holds — without changing the record. To apply it, press Modify and then OCR.

The number is proposed **exactly as the card prints it**, letters and dashes included — a Triple-S contract keeps its `ZUA` prefix, a First Medical contract keeps its dash. If your practice keys a payer's numbers differently, edit the field before saving.

What it reads is a proposal: a digit can be misread on a worn card, and a card Mp10 does not recognise is read with general rules that find the right number less often. Always compare the yellow fields with the card.

6.6.1 Cards that were scanned as a whole page

Some older card pictures are a whole scanner page with a small card on it. When you highlight such a card, the line under the tabs says so. Press **OCR**: Mp10 cuts the card out of the page, shows it, and asks whether to store the card alone. Answer **Yes** and the stored picture is replaced at once, with no other change to the record.

6.6.2 For the administrator

The reader runs on the workstation; nothing is sent anywhere. Which cards it recognises, and where the number sits on each payer's card, is described in `CardTemplates.ini`, which ships through the updates folder next to the program and can be edited at the site when a payer changes its card. The scan resolutions are system settings: **SCANNING / CARDS_DPI** (300) for insurance cards, **SCANNING / ID_DPI** (600) for the patient ID, and **SCANNING / ID_STORE_PX** (1650), the size the stored ID is capped at.

6.7 Checking eligibility

Before the visit, ask the payer whether the patient is covered. In the patients grid, highlight the patient and click **Eligibility** in the side panel, or click the 271 icon in the *Insurance* column.

The *Insurance Eligibility* window lists the answers already received. Click **Fetch New Eligibility record.** to ask the primary payer now. Mp10 connects to the clearinghouse (*Connecting with clearing house. Stand by*) and adds the answer to the list.

If the payer's answer differs from what you have on file — a different spelling, a different date of birth — **Fix patient record with data from Eligibility.** shows the patient record as the payer has it, with the differences highlighted. **Accept** saves those changes; **Cancel** leaves the record alone.

Your administrator can also have Mp10 ask automatically each time the primary member number changes.

7 Encounters

An **encounter** is one visit: one patient, one date, one payer. Everything that happens during the visit hangs from it — what was ordered, the patient's signature, the questionnaire, and in the end, the claim. Mp10 numbers encounters `EN` followed by the year and a sequence, such as `EN26-00012345`.

7.1 Creating an encounter

1. Open the patient (and save it, if it is new).
2. Go to the **Encounters** tab.
3. Click **New** (*Insert New Case*), or press Insert.

The encounter window opens over the patient. Its tabs are **Encounter Data**, **Remarks**, **Orders** and **Questionnaires**.

On **Encounter Data**:

Field	What to enter
Encounter Number	Filled in by Mp10 when you first save
Encounter type (the list at the top)	The kind of visit, from your practice's list
From	The date of the visit. **Tick the box and pick the date** — a new encounter starts with no date
Thru	The last day, for a visit that spans more than one
Referring Phys	The referring doctor. Use the search button; only active doctors are offered
Insurance	The payer for this visit. It starts as the patient's primary card
Inactive encounter	Tick it for a no-show (below)
Accident	*Is Auto Accident*, *Work related Accident* or *Is Other Accident*, and the accident date, when the visit is the result of one

The referring doctor here becomes the referring doctor on the claim, and the accident boxes are sent on the electronic claim, so fill them in carefully.

Click **Save**. Mp10:

- refuses if there is no date (*Encounter date cannot be blank*);
- warns if the patient already has an encounter on the same date, and asks whether to continue — answer **No** if this is a duplicate;
- gives the encounter its number.

The window stays open after Save, so you can go on to the orders. **Exit** closes it.

If Save says *This encounter is open somewhere else*, another user has the same encounter open. Ask them to close it, then press Save again; nothing was lost.

From the patient's Encounters tab you can also print the encounter form (**Print Case sheet**) and a label (**Print Case Label**).

> **Institutional:** in Patients10 the encounter window has the tabs **Encounter**, **Other Information**, **DRG/APC** and **Remarks**. The encounter type is required, and you can enter the pre-authorization, admitting diagnosis, department, source, weight and height. The encounter window prints its own form and label. Orders are entered from the Encounters grid, not from a tab (next chapter).

7.2 Encounters made from the calendar

In the appointment calendar (MpXtrmCal10), an appointment for someone who is not yet a patient has a **Create New Patient Record** button. It opens a new patient window filled in from the questionnaire the patient answered on-line, and when you save the patient, the appointment is linked to the new record.

The calendar does not create encounters. When the patient arrives, open them (the **...** button beside *Medical Record* on the appointment) and create the encounter on their Encounters tab. Once the encounter exists, the calendar shows the appointment as arrived.

7.3 Encounters dated in the future

You can create an encounter ahead of time, for a scheduled visit. The Billing dashboard shows these as *Upcoming encounters pre-created* and does not count them as unbilled.

7.4 No-shows: inactivating an encounter

When the patient does not come, do not leave the encounter as it is: it would sit on the unbilled list for ever. Open it, tick **Inactive encounter** and save. Inactive encounters are shown in grey and left out of the unbilled lists and the dashboard counts.

7.5 The Questionnaires tab

On-line questionnaires the patient filled in before the visit are linked to the encounter when you save it, by the patient's record number or cell phone and the appointment date. The tab lists them; **Open** shows one. You can also send the patient a text message asking them to fill one in.

8 Orders: what the patient came for

An **order** records what the patient is in for — the study, test or procedure — picked from your practice's catalogue. The *Mp10 Orders* guide explains orders in full: setting up the catalogue, labels, sending orders by HL7, results coming back. This chapter covers entering them on the encounter and what they do for billing.

8.1 Why the front desk should enter them

Orders are how the front desk tells the biller what happened. When the biller creates the claim for an encounter that has orders, **the claim's procedure lines start from them**: one line per procedure code, dated on the day of the visit. The biller confirms them instead of reconstructing the visit from paperwork.

This applies to every kind of visit, not only to studies. If your catalogue has an item for a follow-up visit, an injection or a consultation, ordering it puts its code on the claim. Ask your administrator to add an item to the catalogue for every service the practice bills.

Orders also fill the **Ordered** column of the unbilled encounters list, so the billing backlog reads as work — "MRI KNEE RT, FOLLOW-UP" — not as a list of names.

8.2 Entering orders on the encounter

1. Save the encounter once — the Orders tab works only on a saved encounter.
2. Go to the **Orders** tab.
3. Click **New Order** (or press Insert).
4. In the *Item* cell, type the item code, or click the cell's button to pick from the catalogue (the *Pick* list shows each item's description and body part).
5. Add as many orders as the visit needs.
6. Click **Save**.

Delete Order removes the highlighted order when you next save. **Print Label** prints a label for a saved order.

When you save, Mp10 warns if the encounter now has the same study ordered twice, or if the payer has already refused this item for this patient. The orders are saved either way; the warning is for you to act on. The *Mp10 Orders* guide explains both warnings.

The patient's Encounters tab, and the Encounters and Unbilled grids, also have an **Orders** link that opens a separate *Orders for* window over the same orders. It appears only when your practice has orders switched on.

> **Institutional:** in Patients10, open the **Encounters Table**, highlight the encounter and click **Orders** in the side panel. On institutional claims the procedure lines come from the charges posted to the encounter, not from the orders.

9 The patient's signature on the encounter

Besides the HIPAA signature, which is taken once, many practices have the patient sign at every visit: a consent, a financial agreement, an acknowledgement. That signature belongs to the encounter.

9.1 Taking it

1. Open the patient and go to the **Encounters** tab.
2. Highlight the encounter.
3. Click the pen, **Pickup Patient Signature using signing pad**.
4. The patient reads the text on the pad, page by page, taps an option if one is offered, signs and taps **OK**.
5. Click **Accept** on your screen.

The signature shows in the *Signature* column of the encounter list. As with the HIPAA signature, Mp10 keeps who took it, when, the option the patient picked, and the exact text shown. **Signing again replaces the earlier signature.**

If the patient signed the on-line questionnaire before the visit, that signature is copied onto the encounter when you save it, unless the encounter already has one.

9.2 Showing your own text on the pad

What the patient reads before signing is the practice's own text, set once by the administrator in the system registry under **TOPAZ / Encounter-Signature-DisplayMessage**. It can be:

- **plain text** — the pad shows it as it is, wrapped and paged;
- **the path of a text file** — the file is read each time, so the text can be changed without touching the registry;
- **a JSON setting** — which also sets the pad model, the font and size, a list of options for the patient to choose from (for example *I agree* / *I do not agree*), and whether the options and the signature start on a page of their own.

The text can include the patient's e-mail address with `^email`, so the patient can confirm it while signing.

The *Mp10 Web — Signature Pads* guide, chapter *Using it*, gives the format with examples, and the *Mp10 — Signature Helper* guide covers every option. The same setting is used by the Windows programs and by Mp10 Web.

9.3 Finding encounters with no signature

The dashboard's *Encounters missing something* card counts recent encounters with no patient signature. The Billing dashboard's *Needs attention* card shows unsigned encounters from the last seven days.

10 Payments at the front desk

Copays and deposits taken at the desk are posted with **Pat Payment** in the patients grid's side panel (*Post patient payment*). The *Patient Payment Dialog* opens on the **New Payment** tab.

1. Choose the encounter in *Enc. Num.*, or the claim in *ClaimKey* if one already exists. A patient with no encounter cannot be given a payment: create the encounter first.
2. If the patient pays by check, type the check number in *Chk Num.*
3. On each line, choose what the payment is for in *Concept*, type the amount — in *Deductible* if it counts toward what the patient owes on the claim, in *NonDeductible* if it is for something else, such as materials — and the *Method* (Cash, Check, ATH, Visa, M/C, Amex).
4. Click **Save**, then **Print** for the receipt.

All the lines saved together share one receipt number. The **Previous** tab lists the patient's earlier payments and reprints any receipt.

A payment taken before the claim exists is held on the encounter, and the claim shows it as *Recvd @ Pay Wndw*. Until the claim takes it over, the claim carries the tag **Unattached Patient Payments**.

11 Encounters that have not been billed

An encounter with no claim is a visit nobody has been paid for. The longer it waits, the closer it gets to the payer's filing deadline.

In PrBill10, the **Unbilled encounters** button on the toolbar (or *Tables* → *Unbilled Encounters*) lists them. The dashboard's Billing card shows how many there are in the last 90 days and the date of the oldest; click that line to open the list for those 90 days.

The list shows active encounters with no claim. Its columns include the patient, the date, the doctor, the payer, and **Ordered**: what the patient came for, from the orders.

- The *Filter* panel narrows it by encounter type, payer and dates. **Changes apply when you click Refresh.**
- **Show Inactive Encounters** includes no-shows; **Show Only Unbilled**, unticked, shows billed encounters too.
- **Click the *Date* or *Insurance* header to sort by it.** Those two headers carry the sort image; the sorting one turns red. The list opens sorted by date, and the order you pick is kept when you click Refresh. Sorted by *Insurance*, typing in *Incremental Search* jumps to that payer; sorted by date, typing does not search.
- **Auto-claim** (PrBill10; its icon is a form with a lightning bolt) creates claims for all the highlighted encounters at once from their orders and reports (*Auto-claim* in Part II).
- **New Claim** in the side panel creates the claim for the highlighted encounter (Part II). After you save the claim, click Refresh and the encounter leaves the list.

The *Mp10 Dashboard* guide, chapter *Unbilled encounters*, explains how to work through the list and when to inactivate rather than bill.

For a printed list, use *Reports* → *Unbilled Report*.

> **Institutional:** Patients10 has no unbilled list of its own; use *Reports* → *Unbilled* there, or the Unbilled Encounters list in MpBill10.

Part II — Claims, coding and collections

Turning an encounter into a claim, coding it, the error tags, sending it, and following it with the status flags until it is paid.

12 Creating a claim

A claim is the bill sent to the payer for one encounter. Mp10 numbers claims `CL` followed by the year and a sequence, such as `CL26-00012345`.

12.1 Where to start

Claims are created from the encounter they bill:

- **From the unbilled list** (the usual way): highlight the encounter and click **New Claim**.
- **For many encounters at once** (PrBill10): highlight them in the unbilled list and click **Auto-claim** — see *Auto-claim* below.
- **From the Encounters grid** (*Tables* → *Encounters*): highlight the encounter, click **New Claim**.
- **From the patient**: on the patient's **Claims** tab, click **Create New Claim** (or press Insert), then choose the encounter in the claim's *Encounter* list.

The Claims grid itself has no New Claim button: a claim always starts from its encounter. Only BillingUsers and Administrators can create claims.

12.2 What is filled in for you

A new claim starts from what is already known:

- **Insurance** and **Card Number** — the patient's primary card.
- **Comp Ins** — the patient's secondary card's payer, if there is one.
- **Serv/Date** — the encounter's date.
- **Referring Phys** — the encounter's referring doctor.
- **Facility** — your practice's facility.
- **Procedure lines** — one line for each procedure code ordered on the encounter, dated on the visit date, with one unit, pointing at the first diagnosis.

Lines that come from the orders are priced from your fee schedule for the claim's payer, as of the date of service: fee, expected amount, deductible, description, place and type of service. A code the schedule does not carry for that payer stays at 0.00 — look at the *Fee* total before saving, and price any such line by picking the code in its *BillCode* cell.

12.3 Auto-claim

In PrBill10, the unbilled list can build the claims for you. Highlight the encounters (click, then Ctrl-click or Shift-click for more; with none highlighted it takes the current row) and click **Auto-claim** in the side panel. Only BillingUsers and Administrators see the button.

For each encounter Mp10 builds the same claim you would, from what is already recorded: one line for each procedure code ordered (a second order with the same code adds no line), priced from the payer's fee schedule on the date of service; the payer your past claims for that insurance used; the patient's card number; the diagnoses already on the encounter, plus those the order's report confirms. Then it runs the checks that run when you save a claim, and saves it. The claim is saved whole or not at all.

When the batch ends, a summary lists every encounter with its **Result** and the **Claim / Reason**. Double-click a created row (or select it and click **Open claim**, or press Enter) to open that claim; on a skipped or failed row, which has no claim, **Open claim** is greyed out. **Report** opens the encounter's report; it is greyed out on encounters without one, and an encounter with several opens its list of reports to pick from. **The summary stays open**, so you can open and check each created claim in turn; close it with **Close** when you are done. While you work in another browser the summary hides, and it comes back when you return to the unbilled list. Closing the unbilled list closes it too, and the next Auto-claim replaces it. The unbilled list is refreshed as soon as the batch ends: the encounters that got a claim leave it.

An encounter is skipped, with nothing saved, when:

Reason	Meaning
`no orders`	The encounter has no orders to bill
`claim exists`	The encounter already has a claim (primary or any other)
`encounter inactive`	The encounter is marked inactive (a no-show)
`no insurance`	The encounter has no insurance and the patient has no primary
`payer unclear`	The recent claims of all patients for this encounter insurance do not show one payer to bill, or there are none
`payer not on patient`	The payer is not among the patient's cards, so there is no member ID
`no service date`	The encounter has no date
`possible duplicate of <ClaimKey>`	The same patient, date and payer already have a claim with one of these procedures
`no fee for <cpt>`	The payer's fee schedule has no price for that code on that date

A result of **Failed** means something went wrong while building the claim (a lock held by another user, for instance); the reason is shown, nothing was saved, and the encounter can be tried again.

A missing report is not a reason to skip. The claim is created anyway:

- If the encounter has no ICD10 at all, the result reads **Created - no ICD10**. The claim carries the *Missing ICD10s* tag and its lines point at no diagnosis; code it before sending.
- A diagnosis the report does not confirm is simply left off.
- So is a right, left or bilateral diagnosis whose side the line's RT/LT modifier or the report's findings do not name: a right-knee report never adds a left-knee or bilateral code.
- Codes are added only while the claim has fewer than 12, the most an 837 claim carries.

Every claim made this way carries the tag **Auto-created - review**. It means a person has not checked the claim yet. Open the claim, check the payer, the lines and the diagnoses, correct what is wrong, then remove the tag with its **x** and click **Save**. Removing the tag and saving is what records the claim as verified. Leave the tag on a claim you have not checked. The tag does not stop the claim from being sent; check it before you include it in an 837 batch.

An auto-created claim has no performing physician. When its payer is one that refuses claims without one, it also carries **Missing Performing Physician**: fill in *Perf Phys* before you send it.

12.4 The claim window

Its tabs, in PrBill10, are **Summary**, **Procs** (procedures), **ICD10**, **Rmrks** (remarks), **Pymnts** (payments), **EncReps** and **PaprWrk** (paperwork). The procedure lines appear on both Summary and Procs; they are the same lines.

ICD-9 codes were deprecated when billing moved to ICD-10, so the old **ICD9** tab no longer shows. It appears only on a claim from before that change (date of service before 10/01/2015), or on a later claim that already has ICD-9 codes saved on it, so those codes stay visible.

The ICD10 tab shows a check mark when the claim has diagnoses and a red **X** when it has none.

The Summary tab holds the claim's header:

Field	What it is
Medical Record, *Encounter*	The patient and the encounter. Fixed when the claim was started from the encounter
Insurance	The payer billed
Comp Ins	The complementary (secondary) payer, billed after this one pays
Card Number	The member ID. The **270** button beside it asks the payer for eligibility now
Facility	Where the service took place
Pre-Auth Numb.	The prior authorization number, sent on the claim
Perf Phys	The performing (rendering) physician
Referring Phys	The referring physician
ICN	The payer's number for this claim, needed to correct it later — see *Correcting and resubmitting a claim*
Serv/Date, *Bill Date*	Date of service and the date billed
The money fields	Payments, adjustments and balances. Worked out by Mp10; you cannot type in them

At the bottom: **Save**, **Exit** and **Print**, and the claim's **tags** (see *Error tags*).

> **Institutional:** MpBill10's claim window adds the tabs **UbCds**, **Rec Room** and **Charges**. Its lines come from the charges posted to the encounter, an encounter number is required, and the pre-authorization comes from the encounter.

12.5 Primary and secondary claims

The claim you create is always the **primary** claim. You do not create the secondary claim yourself. When the primary payer's payment is posted and part of the balance is assigned to the complementary payer, **Mp10 creates the secondary claim automatically**, billing exactly what was assigned. See *Complementary insurance claims* in Part III.

A secondary claim opens with the bar *Complementary Insurance Claim* and does not let you change its lines.

13 Coding the diagnoses (ICD-10)

13.1 Why ICD-10

HIPAA has required ICD-10-CM diagnosis codes for services on or after **10/01/2015**. The **ICD10** tab is on every claim, and its codes are the diagnoses the payer receives on the electronic claim (837).

13.2 The ICD10 tab

The **ICD10** tab (Alt+C) lists the claim's diagnoses in order. **The first diagnosis is the principal one** — the reason for the visit.

- Type a code straight into the *IcdCode* cell, with its dot (`M17.11`). If the code is valid, its description fills in. If it is not an exact code, the row turns **red** and the description of its category is shown instead.
- **New** adds a row. Delete removes the highlighted one.
- **Move up** and **Move Down** change the order.
- **Run Analizer** checks every code on the claim (see *Run Analizer* below).

The diagnoses belong to the **encounter**, not to one claim: if an encounter has more than one claim, they share the list. Up to twelve are sent on the electronic professional claim; any after the twelfth are not.

13.3 Suggestions and searching by words

An empty row's *Description* cell shows the gray text *Type keywords, e.g. left kidney stone*. That text is only a hint; it is never saved.

Suggestions. When you enter the ICD10 tab, or move to an empty row, a list opens under the grid: *Often billed with* the claim's procedure codes, the diagnoses your practice has billed most often with those procedures. Each one shows how many claims carried it (*billed 12x*). Codes already on the claim are left out. If every row already has a code, Mp10 adds an empty row at the bottom and puts the cursor there; an empty row is never saved. The suggestions are only offered. Nothing is coded until you pick one.

Searching by words. Type words in the *Description* cell, such as `left kidney stone`. After three letters, the list switches to the codes that match and narrows as you type. Each match shows why it matched (the code's description or an entry of the CMS Index to Diseases) and how often your practice has billed it. Typing the start of a code in the *IcdCode* cell (`N20`) lists the codes that start with it.

- **Down arrow** moves from the cell into the list.
- **Enter** or a **double-click** picks the highlighted code. It fills the row as if you had typed the code, the list closes, and the next empty row offers suggestions again.
- **Esc** closes the list and keeps what you typed.
- **No matches:** Enter offers to open the Diagnosis Table (next section).

Rows are one line high unless some row in the list has a second line (why it matched, or an *Excludes1* warning); then every row is two lines. While the list is open the grid's totals row, if it has one, is hidden.

The same list works on the encounter coding screen.

The suggestions are recounted every night, from the primary claims of the last 24 months. A new practice, or a procedure code that has never been billed, gets no suggestions; searching by words still works.

13.4 Looking codes up in the Diagnosis Table

Click the small button inside the *IcdCode* cell to open the **Diagnosis Table**, the ICD-10 encoder. It is one window: a search box over the code tree on the left, the selected code's card on the right with a list of codes under it, and the codes *On this claim* along the bottom.

Nothing you do here reaches the claim until you press **Done**.

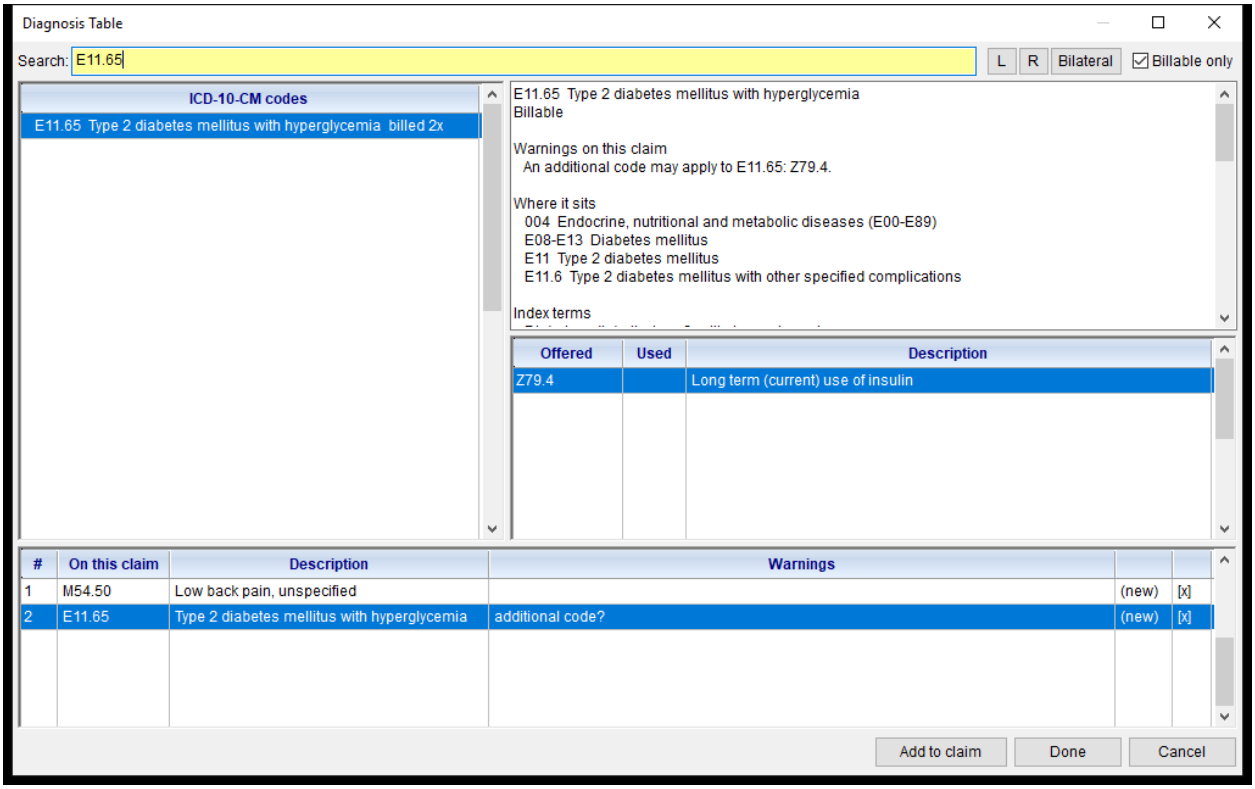


Figure 1: The Diagnosis Table with two new codes on the claim. The first has no warning; the second has a reminder, and the code it offers is in the list under the card

13.4.1 Step by step

Try these on a claim. Press **Cancel** at the end and nothing changes. The results are ordered by how often your practice bills each code, so your list may be in a different order from the one described here.

Open the window

1. Open the claim and click the **ICD10** tab (Alt+C).
2. Click the *IcdCode* cell on the empty row. If the small **...** button does not show inside the cell, press Enter on the cell.
3. Click **...**.

The window opens with the cursor in the *Search* box. It starts on the claim's first code; on a claim with no codes yet, it starts at the top of the tree with an empty *On this claim* list.

Type and add

1. In *Search*, type `low back pain`. Press nothing else: after a short pause the list changes by itself.
2. Click `M54.50  Low back pain, unspecified`. The card on the right shows that code. Read it.
3. Click **Add to claim**.

`M54.50` appears under *On this claim*, marked *(new)*, with an **[x]** beside it. The cursor goes back to *Search*, so you can replace the text there with your next search.

Paste words from a note

1. In the report or note, select a few words, such as `pain in knee`, and copy them with Ctrl+C.
2. Clear the *Search* box and click in it.
3. Press Ctrl+V, or right-click and choose **Paste**.

The list follows, the same as when you type.

Put one side first

1. With `pain in knee` still in the box, click **R**. `M25.561  Pain in right knee` and the other right-side codes move to the top. Nothing is hidden.
2. Click **R** again to turn it off.

Use only the keyboard

1. In *Search*, type the code you already know: `E11.65`.
2. Press Enter, or the Down arrow. The cursor moves to the list.
3. Press Enter on the code. It is added.

Double-click

1. In *Search*, type `hypertension`.
2. Double-click `I10`. It has nothing under it, so it is added.

A double-click does different things depending on the row:

You double-click	What happens
A billable code with nothing under it	It is added
A row with a + in the tree	It opens. Nothing is added
A gray code with nothing under it	Nothing is added. The line beside the buttons says why

Find a code in the tree

1. Clear the *Search* box. The tree comes back.
2. Double-click `E00-E89`, then `E08-E13`, then `E11`, then `E11.6`. Each one opens.

3. Click `E11.65` to read its card, then click **Add to claim**.

Choose from the list under the card

1. In the tree, click a gray code that has codes under it, such as `E11.6`. The **More specific** list shows the codes you can bill.
2. Click one of them. The card changes to that code.
3. Click **Add to claim**. The code you clicked in the list is the one added.

A code that needs a seventh character works the same way. In the tree `S72.001` is one row: **Add to claim** refuses it and says why, and the list under the card has `S72.001A` and the other finished codes. If you search for `S72.001` instead, the results list the finished codes directly.

Take back a code you just added

- Click its [**x**].
- Or click its row in *On this claim* and press Delete.

Only codes marked *(new)* can be taken back here. A code that was already on the claim has no [**x**].

Follow a warning

Add `E11.65`. The *Warnings* column says *additional code?*, in black with no **!**. That is a reminder:

1. Click the `E11.65` row in *On this claim*. The list under the card is now headed *Offered*, and shows `Z79.4  Long term (current) use of insulin`.
2. If the patient uses insulin long term, click `Z79.4` and click **Add to claim**. If not, leave it.

Now search for `E10.9`. Its row in the results starts with **!**, and it says that `E10.9` and `E11.65` are never coded together. You can still add it, and both rows then say *Excludes1 with…* in red. If that was a mistake, click the [**x**] on `E10.9`.

Add `M25.569`, and the column says *side not specified*. Click its row, click `M25.561` or `M25.562` in the *Offered* list, and click **Add to claim**. The new code goes on as another row, so remove `M25.569` with its [**x**]. If `M25.569` was already on the claim, delete it on the ICD10 tab after **Done**, and check the *Diag* on each line.

Finish

1. Click **Done**. The window closes with no questions.
2. On the ICD10 tab, the first new code fills the empty row at the bottom, and any others follow it.
3. Click **Save** on the claim. Each code has the number the window showed beside it. That is the number to use in a line's *Diag*.

To leave without changing anything, click **Cancel**, click the window's close box, or press Esc.

Pick one code for an encounter

1. On the encounter, click the search button beside the diagnosis field.
2. Type your search, and click the code.
3. Click **Use this code**. The field fills in and the window closes.

An encounter dated before October 2015 opens the old ICD-9 list instead.

Keys

Where	Key	What it does
Search box	Enter or Down arrow	Moves to the list
List or tree	Enter on a code with nothing under it	Adds the code
Tree	Enter on a row with a +	Opens the row
On this claim	Delete	Takes back a new code
Anywhere	Tab	Moves to the next part of the window
Anywhere	Esc	Closes the window without changing the claim

13.4.2 Finding a code

- **Search:** type words, or the start of a code, in the box. The list changes by itself after a short pause, from three characters on. It shows codes that can be billed on the claim's date of service, the likeliest first. Clear the box and the tree comes back, open where you left it.
- **Nothing found:** the list shows the codes that only mention your words, under a heading that says so. Some of them cannot be billed.
- **L, R, Bilateral:** puts that side's codes first in the search results and hides nothing. Click it again to turn it off.
- **Browse:** the tree goes chapter, section, category, then each code. A row with a + opens with Enter or a double-click. Gray rows cannot be billed on the date of service. *Billable only*, on when the window opens, hides the gray rows with nothing billable under them; it applies to the tree, not to search results.

A code that takes a seventh character is one row. Its finished codes are in the list under the card.

13.4.3 The code's card

The card follows the row you select. It says whether the code can be billed on the date of service, and why not if it cannot; where the code sits in ICD-10; and its notes in full, saying which code each one comes from (*from E11*). It also shows the entries of the CMS Index that lead to the code, how often your practice has coded it, and the procedures it is billed with.

When the claim's payer has refused this code with one of the claim's procedure codes, *Payer history for this claim's lines* comes before the rest of the card, after any warnings. A part with nothing to say is left out.

The **More specific** list has the billable codes under the selected one, or the other choices for an unspecified code, most used first. Click one to read its card.

13.4.4 Adding codes

Add to claim adds the code on the card. Enter or a double-click does the same on a code with nothing under it. A code that cannot be billed on the date of service, or that is already there, is not added; the line beside the buttons says why.

On this claim lists the claim's codes in order, then the ones you added, marked *(new)*. The number on each row is the one it will have once saved, which is the number a line's *Diag* uses:

- **New codes** can be removed: click their **[x]** or press Delete.
- **Codes already on the claim** cannot be moved or removed here. A procedure line points at a diagnosis by its position, so that is done on the diagnosis list: the ICD10 tab on a claim.

13.4.5 Warnings

The *Warnings* column says what to look at on each code, judged over the whole claim for its date of service. It follows every code you add or remove.

Click a row and its card lists the warnings in full. Codes you can add go to the top of the list under the card, headed *Offered*: click one, then **Add to claim**. A code named by a warning that is already on the claim, such as the other half of an Excludes1 pair, says *Related, on this claim.* Click it to read its card; it cannot be added again.

A warning informs. It never stops you adding a code or pressing **Done**.

Adding a code does not replace another. The code you add goes on as a new row at the bottom. A code already on the claim stays where it is, and each line's *Diag* still points at its position. To be rid of it, press **Done** and delete it on the diagnosis list: the ICD10 tab on a claim, the screen's own list on an encounter. On a claim, then check every line's pointers: see Pointing lines at diagnoses.

The column says	What it means
not billable	The code cannot go on a claim for this date: a header code that has more specific codes under it, retired, not yet valid, or not an ICD-10-CM code. Add the right code, then delete this one.
needs 7th character	The code is not finished. Add the finished code from the list, then delete this one.
additional code?	ICD-10-CM says "use additional code". Add one only if it applies. Some common codes say this every time, so on its own it is in plain black with no **!**: a reminder, not a fault.
Excludes1 with…	ICD-10-CM says the two codes are never coded together. It shows on both. Keep both only if the two conditions are unrelated.
order with…	A "code first" instruction asks for the two codes the other way round. It shows on both. Change the order on the diagnosis list.
side not specified	The code leaves the side open, and codes for the side exist. Add the right one, then delete this one.
patient sex	The code does not fit the sex on the patient's record. Shown when you open the window from the code cell's button.
? checks did not run	The checks could not be read, and the line beside the buttons says so too. An empty column would not mean a clean claim here; close and reopen the window.

Every warning but that reminder is in red, with a **!**. A code you just added and no longer want goes with its **[x]**, with nothing else to do.

Old problems show too. A pair saved on the claim long ago has its warning on both rows, though you added neither.

13.4.6 Closing

Done adds the new codes to the bottom of the claim's list, with no questions. The first one fills the empty row there, if there is one. An empty row between two codes stays as it is, because filling it would renumber the codes under it. Then press **Save** on the claim as usual. **Cancel**, Esc and the window's close box leave the claim as it was.

The encounter coding screen uses the window the same way. The diagnosis button on an encounter or a discharge opens it to pick one code: there is no claim list and no warnings column, and **Use this code** fills the field and closes. **Tables > ICD10** opens it only to read: no adding, and Esc closes it.

13.4.7 Run Analizer

Run Analizer, on the ICD10 tab, does not open this window. It checks every code on the claim and asks you, one message at a time:

- **A code is not billable** — *ICD10 Code seems invalid*, with the reason. Choose the more specific code.
- **An additional code may apply** — tick the ones that apply.
- **A seventh character is needed** — pick it from the list.
- **Patient sex** — the patient's sex does not fit a code.
- **Excludes1** — two codes are never coded together. Keep both only if the two conditions are unrelated.

- **Diagnosis sequence** — a "code first" instruction asks for the codes the other way round. Move them.

13.5 Pointing lines at diagnoses

Each procedure line has a **Diag** column: which diagnoses justify it, by their position in the ICD10 list. `1` is the first diagnosis, `1,2` the first and second. Separate numbers with commas.

Two things to watch:

- **Picking a code on a line resets its Diag to 1.** Set the pointers after the codes.
- **Reordering or deleting diagnoses changes what the numbers mean.** After moving or deleting a code, check every line's pointers.

13.6 Warnings on the encounter coding screen

When one person codes in Patients10 and another bills, a coding problem used to surface only when the biller saw an error tag on the claim and went back to the coder. The **Coding** screen of an encounter now shows the problem to the coder, in its own *Warnings* column, while the case is open. The codes here are the ones the claim uses, so a correction here is a correction on the claim.

The column uses the same words as the Warnings column of the Diagnosis Table, in red with a **!** for anything but a reminder. It is judged when the screen opens, and again after every code you type or pick, delete or move, and after the Diagnosis Table or the AI coding button adds codes.

To read a warning in full, double-click its cell, or move to it with the arrow keys and press Enter.

A pair. Type `E11.65`, press Enter, then type `E10.9` on the next empty row and press Enter. Both rows turn red: *! Excludes1 with E10.9* and *! Excludes1 with E11.65*. Delete the code that does not belong and the other row clears at once. If the two conditions are truly unrelated, keep both.

An order. With `N18.6` above `I12.0`, both rows say *! order with…*. Click the `I12.0` row and press **Move Up**, and the warning clears.

Save. If something is still in red, **Save** saves your codes and then lists what to check. It never refuses. It does not repeat the same list while nothing has changed, and the column keeps showing it.

Two limits:

- **Checks that need the claim's lines stay on the claim.** *Wrong Side ICD10*, *Bilateral CPT, One-Side ICD10*, *ICD10 Relation* and *Denied Before* judge the codes against procedure lines and a payer, which the coding screen does not have.
- **A tag already on a claim stays until the claim is saved again.** After the coder corrects the codes, the biller opens the claim and presses **Save**. The tag then clears by itself if the problem is gone.

14 Coding the procedures

14.1 The procedure lines

Each line is one service:

Column	What it holds
BillCode	The procedure code, followed by its modifiers
Unts	Units (at least 1)
Fee	Your charge
Expected	What the payer is expected to allow — this is what the claim bills
Deduct	The patient's deductible share
From, *Thru*	Dates of service
Diag	The diagnosis pointers (above)
Tos, *Pos*	Type and place of service
Performing	The performing physician for the line

Add a line with **New** (*Add a procedure line*), Insert, or the down arrow on the last line. Delete removes the highlighted line straight away.

14.2 Codes, prices and the fee schedule

Type the code in *BillCode*, or click the cell's button to pick it from **Valid Billing Codes** — the codes your practice has priced **for this claim's payer**, as of the date of service. When you pick or confirm a code, Mp10 fills in *Fee*, *Expected*, the description, the revenue code, the place and type of service, and the deductible.

Valid Billing Codes opens only when you ask for it: the cell's button, or Enter on the *No matches* row of the list under the grid. It no longer opens by itself when you leave an empty cell or type a code the payer has no price for. It lists the codes billed most often on this payer's fee schedule at the top. Its *Billed* column shows how many claim lines used each code in the last 12 months. Typing a code still jumps to it, and clicking the *Cpt Code* header sorts the list by code.

Prices come from your practice's billing codes table, kept by payer and by effective date in MpAdmin. If you type a code that is not priced for the payer and press Enter, the cell stays open and the list under the grid says *"76999" is not on the MCS fee schedule*, with the priced codes that start the same way, so you can correct it or pick one. If you leave the cell instead, the line keeps the code it had, and you hear a beep. If the code is not on the fee schedule at all, ask whoever keeps the fee schedule to add it.

A code with a modifier the fee schedule does not list is still accepted and priced from its first five characters: `76881 XY` takes the price of 76881.

Changing the units changes *Expected* and *Deduct* in proportion.

14.3 Finding a code by words

You don't need to know the code. Type words in the *BillCode* cell, such as `ext sono` or `lap chole`. After three letters, a list opens under the grid with this payer's priced codes that match, best first. The list narrows as you type.

The words are matched against every description your practice has used for a code: the fee schedules of all your payers and the descriptions on past claim lines. That is why abbreviations and Spanish wording that your office has used find the code. When a code matched through one of those other wordings, its row shows it on a second line: *as billed: EXTREMITY SONOGRAM LTD.*

The list has four columns: *Code* (with its modifiers), *Description*, *Billed* (how many claim lines used the code on this fee schedule in the last 12 months; blank if none) and *Fee* (this payer's price). Rows are one line high, unless some row has an *as billed* line; then every row is two lines. Codes whose modifier you typed rank first: `sono rt` puts the RT codes on top.

Typing the start of a code (`7688`) lists the priced codes that start with it. A full code followed by Enter works exactly as before.

- **Down arrow**, or **Enter** after words, moves from the cell into the list.
- **Enter** or a **double-click** picks the highlighted code. It fills the line as if you had typed the code, and the cursor moves to an empty line at the bottom.
- **Esc** closes the list. The cell keeps the code it had.
- Words are never saved as a code. If you leave the cell without picking, it goes back to the code it had.
- Leaving an empty *BillCode* cell (up arrow, Tab, a click elsewhere) does nothing. The line stays empty.
- **No matches:** press Enter on the *No matches* row to open **Valid Billing Codes**. That and the cell's button are the only ways it opens.

Suggestions. On an empty procedure line, on either the *Summary* or the *Procs* tab, the list opens by itself with the codes most billed on this payer's fee schedule. It does not take the cursor. Press the down arrow to move into it, or type to search instead. Arriving on the *Procs* tab puts the cursor on an empty line at the bottom; an empty line is never saved. Returning to the *Summary* tab leaves the cursor in the claim's header.

On a closed claim the *BillCode* cell works as before, with no list.

Where the list opens. While the list is open, the grid's totals row is hidden to leave room for lines. On the *Procs* tab the list takes the bottom of the grid. On the *Summary* tab, where the grid is short, the grid shrinks to half its height and the list fills the rest of the page, over the balances, dates and facility fields. Those come back as soon as the list closes: after a pick, Esc, or a change of tab.

The counts are refreshed every night. Until the first count after the Data Dictionary update, the list searches the fee schedule's own descriptions only, offers no suggestions, and *Billed* shows 0.

14.4 Modifiers

Type modifiers **after the code, separated by spaces**: `76881 TC RT`. Up to four are sent. If your fee schedule prices the code with its modifiers as a separate entry, that price is used; otherwise the base code's price is. After the price fills in, check the modifiers are still on the line.

14.5 Checking a batch of claims before sending

In the Claims grid, select claims and click **Analyze 837** in the side panel. Mp10 reviews them all and prints a list of warnings: missing or invalid codes, duplicate diagnoses, codes that need a seventh character or a side, missing place of service, the tags each claim carries. Nothing is changed or sent.

15 Saving the claim, and error tags

15.1 When you save

Click **Save**. Mp10:

- refuses a new claim with no lines (*Nothing to save*);
- warns if the same codes, dates and payer are already billed for this patient (*Billing Codes, dates, and insurance seems to be duplicated*) — answer No if it is a duplicate;
- refuses a payer that is not set up in your insurance list;
- gives a new claim its number;
- runs the **claim checks** and tags the claim with what they find.

If anything fails, nothing is saved: *The claim was NOT saved. Everything from this save has been rolled back*, followed by the reason.

15.2 Error tags

A **tag** is a coloured label at the bottom of the claim window naming a problem a check found: *Wrong Side ICD10*, *ICD10 Relation*, *NCCI Pair Needs Modifier*, *Denied Before*, and others. Tags never stop you from saving or sending a claim. They tell you what a payer is likely to reject, before it does.

- **Hover a tag** to see what it found.
- **Click it** for the full explanation: which lines, which codes, and why.
- **To clear it,** fix what it names and save. Each check runs again on every save and removes its own tag when the problem is gone.

Three checks offer to fix the problem for you as you save, and change nothing unless you answer Yes:

- **Insurance DOB Mismatch** — the payer's last eligibility answer gives a different date of birth. Yes writes the payer's date on the patient's insurance card, not on the patient.
- **Insurance Sex Mismatch** — Yes updates the patient's record.
- **Payer Not On Patient** — the claim bills a payer the patient has no card for. Yes adds it to the patient's insurances; check the member number and dates afterwards. Answer No if the claim is the one that is wrong.

Three tags learn from the remittances you post (Part III): **Denied Before**, when this payer has refused these codes often and recently; **Refused For This Patient**, when the payer refused these very codes for this patient and has paid nothing for them since; and **Missing Performing Physician**, when the claim has none and this payer has refused claims sent without one.

You can also add your own tag with the + button beside the tags, to mark a claim for yourself or a colleague.

The full list of checks, what each looks for and how to switch one off is in the *Mp10 Dashboard* guide, chapters *Claims with error tags* and *The claim checks*. The dashboard's *Needs attention* card lists every claim tagged in the last seven days.

In the Claims grid, a claim with tags shows a tag icon in *SrvDate* and its row is coloured orange.

16 Finding claims

Open the **Claims table** from the toolbar (or *Tables* → *Claims*). The *Claims Data Grid* lists every claim.

16.1 The columns

ClaimKey, *Creation*, *MedRec*, *Encounter*, *SrvDate*, *Contract* (the member ID), *Insurance*, *BillDate*, *Name*, the charge (*Real Amt*), insurance paid, adjusted and balance, the deductible, patient paid, balance and adjustments, the user who created the claim and the performing physician.

The *ClaimKey* column shows the claim's **status flag** (next chapter). Right-click it, or press F9, to see the claim's status history.

16.2 The row colours

Colour	Means
Orange	No bill date, no diagnoses, or the claim has tags
Cyan	Resubmitted: corrected, replacement or void
Magenta	Created by AutoTasks, Mp10's automatic service
Grey	No charge or no service date
Blue	Everything else

16.3 Filtering

The *Filter* panel on the left narrows the grid by any combination of:

- **Status** — one flag, or *Missing Flag* for claims with no status at all (no flag, or the *Null* flag 0);
- **Ins** — one payer;
- **Srv From / Srv Thru** — dates of service;
- **Bill From / Bill Thru** — billing dates;
- **Wrk From / Wrk Thru** — the date the claim was created;
- **Med Rec** — one patient;
- **Primary claims only** — leaves out complementary (secondary) claims.

Switch the filter on with its LED. To jump to one claim, sort by *ClaimKey* and press F8.

16.3.1 Filter From File: claims from a list you have

Filter From File, the last link in the side panel, shows exactly the claims on a list you have: a payer's rejection report, a spreadsheet from a collections review, an email with claim numbers in it. The window opens with an empty list; fill it, check it, and click *Run Filter*.

Filling the list:

- **From a file.** Click the magnifier beside *Filename* and pick the file. Every claim number in it is listed, wherever it sits in the file: one per line, several on a line, inside a sentence, in any column of a spreadsheet. PDF, Excel (*.xlsx* and the older *.xls*), Word (*.docx*), text, CSV, tab-delimited, XML, web pages and scanned images (TIFF, PNG, JPEG, read by OCR) all work.
- **PDF files.** A PDF from a payer portal or a clearinghouse holds real text and is read at once. A **scanned** PDF is only pictures of pages: Mp10 reads each such page with OCR, the same reader that fills in insurance cards, which takes a few seconds a page. A window shows which page it is on; *Cancel* stops after that page and keeps the claims already found. OCR can misread a smudged number, so compare the count with the report — a number it cannot read whole is left out, never guessed. If the window says the OCR files are missing, the workstation can still read text PDFs; scanned pages need the card-reading files installed.

- **By pasting.** Copy any text that holds claim numbers, click in the list, and press Ctrl+V. Only the claim numbers are taken from it.
- **By typing.** Press the down arrow on the last row to open an empty one, type the claim number, and press Enter.

A claim number is *CL*, two digits, a dash and eight digits — CL26-00012345. Lower case is fine; it is shown in capitals. Anything else is refused: a typed number that does not have that shape gets a message, and partial numbers in a file (seven or nine digits, say) are skipped rather than guessed. Each claim is listed once, however often the file repeats it, and picking a second file adds its claims to the ones already listed.

Checking the list. The rows are numbered, and the count under the list (*37 ClaimKeys*) tells you how many claims the filter will show — compare it with the report you started from. Select a row and press Del to remove it. *Open File* opens the file you picked in its own program (Excel, Notepad), so you can see what was in it. *Run Filter* is available as soon as the list holds one claim.

Run Filter shows only the listed claims; the *Filter* panel's other settings are ignored while it is on. *Cancel Filter* closes the window and shows every claim again. The window can be made larger by dragging its edge, and opens at that size next time.

For the administrator. Files are read on the workstation; nothing is sent anywhere. PDFs need one file beside the program, `pdfium.dll`, which ships through the updates folder like the other DLLs — put it there before the first program that reads PDFs. Scanned pages also need the card-reading files (`tessdata\eng.traineddata` and its DLLs). A workstation missing either says so when a PDF is picked; every other kind of file still works.

16.4 Doing things with claims

The side panel acts on the highlighted claim (or claims):

Link	Does
Open Claim	Opens it (also Enter or double-click)
Print Claim, **Preview Claim**	The paper claim form, for every selected claim
Post Payment	The payment window for the claim (Part III)
Notes, **Attachments**, **AuditLog**	The claim's notes, documents, and history of changes
Claim Status	The status history; with several claims selected, change their flag together
Eligibility	Asks the payer now; with several selected, asks for each
Analyze 837	Reviews the selected claims before sending
Outbound 837	Builds and sends the electronic claims file
Pat Payment	A patient payment on this claim, with a receipt

Double-click the *Encounter* cell to open the encounter, or *MedRec* to open the patient. Ctrl- or Shift-click selects several claims. Claims are never deleted: to cancel one, adjust it to a zero balance.

17 Sending claims

17.1 Electronically (837)

1. In the Claims grid, select the claims to send. **They must all be for the same payer**; filter by *Ins* first.
2. Optionally, run **Analyze 837** and correct what it finds.
3. Click **Outbound 837**. Mp10 writes the claims file and shows a report of every claim in it, with the file name and any warnings at the end.
4. Answer **Yes** to *Send claims file to clearinghouse*.

Each claim records, in its notes, the name of the file it went out in. That file name is your batch number.

Outbound 837 also collects whatever the clearinghouse has waiting — acknowledgements, claim statuses and remittances — and processes it, so the flags in the grid are up to date afterwards.

Supervisors and Administrators also have *Actions* → *Send Outbound 837 Claims*, which lists the claims files waiting in the outbound folder and sends them, and *Actions* → *Fetch X12 Inbound Responses*, which collects and processes the clearinghouse's files on demand. Normally AutoTasks does both on a schedule; see the *Mp10 AutoTasks* guide.

17.2 On paper

Print Claim in the Claims grid prints the claim form — CMS-1500 or UB-04 — for every selected claim; **Preview Claim** shows it first. *Actions* → *Print Claims* prints all the claims of a date range for the payers you tick. Which form a payer gets is set on the payer's record (*Form #*). A printer whose name contains `FORM` is used without asking.

18 What the payer's record decides

Several things about a claim are decided not on the claim but on the **payer's record** — the insurance company, under *Tables* → *Insurance Companies* (or in the administration program). A biller who knows these settings exist can tell "the system did it wrong" from "this payer is set up that way", and ask for the right change.

On the **Ins Info** tab:

Setting	What it decides
Professional / Institutional	Whether this payer's electronic claims go out as professional (837P, the CMS-1500 content) or institutional (837I, UB-04), and which NCCI tables the claim checks use for it
Form #	Which paper form prints for this payer. A payer with no form number cannot print a paper claim (*Undefined Insurance Form*) and is left out of the professional Unbilled Report
In-active	The payer can no longer be put on a card or a claim: *Insurance is Inactive*. Old claims keep it
Billing Fees From	Institutional only: whether the payer is billed by DRG/APC, private rates or the fee schedule

On the **X12** tab:

Setting	What it decides
Receiver ID, **Submitter**, **Sender** and the ISA/GS fields	The clearinghouse addressing on every electronic claim, eligibility request and remittance match for this payer. Set once by the administrator with the clearinghouse's instructions; a wrong value rejects every claim
Auto-Fetch Patient Eligibilities	Whether AutoTasks asks this payer, by itself, for the eligibility of each upcoming encounter. Off, and eligibility is only checked when someone clicks *Eligibility*
Include PRV Taxonomy segment	Whether the electronic claim carries the provider taxonomy segments. Some payers demand them, some reject them
Check NCCI PTP pairs	**Off by default.** Tick it and the claim checks apply the NCCI procedure-to-procedure edits for this payer, placing the tags *NCCI Pair, Never Together* and *NCCI Pair Needs Modifier*. Tick it for every payer that follows Medicare's NCCI edits — most do. The *Units Over MUE* check runs for every payer regardless

The fee schedule itself — the price and expected amount of each procedure code, by payer and by effective date — is the **Billing Codes** table, kept in the administration program. A line that prices at 0.00 on a claim means the code is not in that table for that payer on that date.

19 Status flags: the rebilling and collections system

Sending a claim is the start, not the end. Payers acknowledge it, reject it, pay part of it or all of it, sometimes months apart. **Mp10 keeps every one of those answers on the claim as its status history, and shows the latest as a coloured flag.** The flags turn the Claims grid into a work queue: filter by flag and you have the list of claims that need you.

19.1 The flags

Flag	Name	Means	Set by
Green	Fully Paid	Nothing left for the insurance to pay	Posting a payment or remittance
Yellow	Partially Paid (also *Pending*)	Something paid, something still owed	Posting; a payer status saying it is pending
Red	Rejected	Refused, by the clearinghouse or the payer, or paid nothing on the remittance	Claim status responses; posting a remittance that pays nothing
Grey	Received by Ins	Accepted by the clearinghouse or payer, not paid yet	Claim status responses and acknowledgements
Black	OverPaid	More was paid than billed	Posting; also a payer status saying the claim was not found
Blue, checkered, pink, purple	User Def 1–4	Your practice's own meanings	You
None	No status	Nothing heard yet	—

The four user flags can be renamed for your practice — *Appealed*, *Called payer*, *Sent to collections* — by your administrator, in the system registry under **BILLING / UserDefined-1-ClaimStatus** to **-4-**.

19.2 Where flags come from

- **Claim status responses (277)** and the clearinghouse's acceptance reports arrive automatically and add a row to the history, with the payer's words. They move the flag only on a claim that has no status, is rejected or is received. They never move a claim that is paid, partly paid, overpaid or carries one of your own flags.
- **Posting a remittance or a payment** works out the flag from the balance: green when nothing is left, yellow when something is, red when nothing was paid, black when overpaid. A payment that settles the claim always turns it green.
- **You**, through the status history or *Change Flags* (below).

19.3 The status history

Press **F9** on a claim in the Claims grid (or right-click its *ClaimKey*, or use **Claim Status**). The *Claim status history* window lists every status the claim has had, oldest first: the flag, the date it was received, what was said, who recorded it and when.

The *Data* column holds the payer's or clearinghouse's own words for status responses, *Remittance Posting.* for remittances, *Payment Posted by user.* for payments keyed by hand. The button in the cell shows it in full, and its **Print** prints a status response as a readable form.

To record a follow-up — a call to the payer, an appeal sent, a promise to reprocess:

1. Click **Add New most recent status to list**.
2. Choose the flag.
3. Set the date.
4. Type a note in *Data* — what was done, who you spoke to, what was promised. A row saved without one reads *Changed by user manually*, which tells the next person nothing; write the note.
5. Click **Save**.

The new row becomes the claim's flag. The flag always follows the row with the latest date, and of rows with the same date, the one recorded last — so a row you date earlier than the latest one already there does not move it. A row you add and leave at *Null* with no note is not saved.

Saved rows cannot be edited, so the history is a true record of what happened when. A row entered by mistake can be removed by the person who entered it:

1. Select the row.
2. Click **Delete** (the button under *Add New*), or press **Del**.
3. Click **Save**. **Exit** instead keeps every row.

You can delete only rows you entered yourself, in this window or with *Change Flags*. Rows from status responses, acknowledgements, payments and remittances, and rows other people entered, stay. After a delete, the flag goes back to the latest row left. A row you added and have not saved yet can always be removed the same way.

To change the flag of many claims at once, select them in the grid and click **Claim Status**. The *Change Claim Status* window lists the flags; tick one and click **Change Flags**. Each claim gets the new flag and a history row *Changed by user manually*. This is also the only way to clear a flag back to no status.

Mp10 has no follow-up date. The date on the history row, and the *Days* column of the Claims Status report (below), show how long a claim has been waiting since its last status.

19.4 Working the queue

A routine that keeps money from slipping past filing deadlines:

1. Rejections, every day. The dashboard's Billing card shows the number of rejected claims in the last 90 days. Click it for the **Rejected claims** list: each claim with the date it was rejected, where the rejection came from — *277/CH* for the clearinghouse or payer, *835* for a remittance — and the reason, in the payer's words or as the remittance's reason codes. Double-click a claim to open it, correct it, and resubmit (next chapter). Click **Refresh** and corrected claims leave the list.

2. Claims with no answer. A claim that has heard nothing at all after a couple of weeks may never have reached the payer. Filter the Claims grid by *Missing Flag* and a range of bill dates, and check those claims. The dashboard shows the count of primary claims with no status served in the last 90 days and the date of the oldest; click it and the grid opens with that filter set.

3. Claims stuck in received or partly paid. *Reports → Claims Status* lists claims with an insurance balance by flag and payer, with **Days** — how long each has sat on its current flag. Tick *Received* and *Pending*, read the oldest first, and call or appeal. Record each call in the status history so the next person knows. The report lists only claims that have a flag; use the grid's *Missing Flag* filter for the ones with none.

4. Dollars by age. *Reports → Aeging of Accounts* totals what is owed by age — 30, 60, 90 days and beyond — for insurance and for patients, by payer or by financial class. Use it to see where the money is waiting, and the Claims Status report to work it.

5. What the patient owes. When the insurance has paid its part, the rest is the patient's: see *Collecting from the patient*.

20 Correcting and resubmitting a claim

When a payer rejects or underpays a claim, correct the claim and send it again, telling the payer which claim it replaces.

1. **Correct the claim.** Open it, fix what the rejection names, save. Check the tags.
2. **Set the submission code.** In the Claims grid, highlight the claim and press **F4**. In *Claim Submission Code*, choose:
   - **Corrected** or **Replacement** — this claim replaces the one the payer has (frequency code 6 or 7);
   - **Void** — cancel the claim the payer has (frequency code 8);
   - **Original Claim** — a first submission (the default).

   Click **Save**. The claim's row turns cyan, and the claim window's bar starts with *Corrected:*, *Replacement:* or *Void:*.
3. **Check the ICN.** A corrected, replacement or void claim must carry the payer's number for the original claim, its **ICN** (on the Summary tab). When the original was paid or denied on an electronic remittance you posted, Mp10 has already filled it in. When you type it yourself from a paper EOB, **type F8 before the number** — `F8` followed by the ICN, with no space — or it is not sent as the original claim's number. Mp10 does not warn you if the ICN is missing; the payer will reject the claim.
4. **Send it** with Outbound 837, or print it.

This is the first reason to post remittances whole: they bring the ICN onto the claim for you. Part III explains the others.

21 Collecting from the patient

Once the insurance has paid, what remains on the claim is the patient's balance, shown in the claim's *Pat Balance* and in the Claims grid's *PatBal*.

Collection letters. *Actions* → *Pat Collection Letters* lists the claims with a patient balance for a range of dates — service dates or billing dates, whichever box you tick — at or above the **Minimum Debt to print collection letter** you enter. For each claim it shows how many days it has been since the last letter, and which letter that was. Tick **Also Print Next Corresponding Letter** to print each patient their next letter: the first letter for a patient who has had none, and the next one in the series once 30 days have passed since the last. Your practice can have several letters, each firmer than the last; how many is set by **BILLING / MAXIMUM_PATIENT_COLLECTION_LETTERS** (three by default), and a patient who has had the last one is not printed again. Every letter printed is recorded on the patient's **Letters** tab.

One patient at a time. On the patient's **Claims** tab, *Current Patient Bal* shows what they owe in total, and **Print Patient Statement** prints a collection letter for them. The patients grid's **Prnt Statement** prints a full statement of every service.

Taking the payment is described in *Payments at the front desk* (Part I), or with **Pat Payment** in the Claims grid, which prints a receipt.

Part III — Remittances and payments

Receiving, reading and posting the payer's remittances, the other ways to enter a payment, and why the remittance is the one to use.

22 What a remittance is

A **remittance** is the payer's explanation of one payment: one check or electronic transfer, and every claim it pays, denies or adjusts. For each claim it says what was billed, what was allowed, what was paid, what the patient owes, and why anything was not paid, in standard reason codes. It also carries the payer's own number for each claim, the ICN.

Mp10 receives remittances in two forms:

- **Electronic remittances (X12 835)** come from the clearinghouse. AutoTasks collects and imports them on a schedule.
- **PDF remittances** (EOB or ERA PDFs downloaded from a payer's website) are saved into a folder that AutoTasks watches. It reads them and imports them.

Both end up in the same place, the **Remittances** window, **as remittances waiting to be posted**. Importing posts nothing: no claim balance changes until someone posts the remittance. Setting up both is described in the *Mp10 AutoTasks* guide, chapters *Electronic remittances: the two kinds*, *X12 835 electronic remittances* and *Electronic remittances (EOB / ERA PDFs)*.

A PDF remittance that AutoTasks could not import safely (a possible repeat, a claim number it could not match, an amount that looks wrong) is not imported and not archived: it waits in the `review` sub-folder of the watched folder, with a short text file saying why. Staff enter it by hand, using the **EOB** tab of *Fetch Claims* (see *Entering a remittance by hand*): its **AutoTasks review folder...** button lists what is waiting there, and the EOB is read, previewed and loaded like any other. Or ask support.

A paper EOB that arrives by mail can be entered by hand in the same window: see *Entering a remittance by hand*.

23 Finding a remittance

Click **Remittances Data Entry Dialog** on the toolbar (the check), or *Tables* → *Remittances*. The window has two tabs:

- **Remittances** — the list of remittances.
- **Detailed Check Tree View** — one remittance, open for posting.

23.1 The list

The **Remittances** tab lists one row per check, with its *Check* number, *Insurance*, *ChkDate* (check date), *RcvDate*, *Deposit* date, *ChkTotal*, *Amount*, *Retained*, whether it is posted, the file it came from and the number of lines. A green dot beside the check number means posted; a red dot means waiting to be posted.

In the **Remittances Period** panel on the left, narrow the list by check dates (*From*, *Thru*), by payer (*Insurance*, any part of the name), or tick **Show Only Posting Pending** for the ones still to post. Then click **Reload Remits** in the *Actions* panel.

To find a check number or an amount, sort by the *Check* or *ChkTotal* column and look down the list.

A check that is partly posted shows twice: one row for the posted lines, one for the lines still waiting. The waiting lines are almost always claims Mp10 could not match (see *When a claim is not found*).

23.2 Opening one

Double-click it, or press Enter. It opens on the **Detailed Check Tree View** tab.

24 Reading a remittance

24.1 The check

The **Remittance Info** panel on the left describes the check:

Field	What it is
Chk Num, *Ins*, *Chk Date*	The check (or transfer) number, the payer, the date
Rcvd On, *Dep On*	When it was received and deposited
Claims Pd	The sum of everything paid on the claims
PLB Adj	Money the payer added or withheld outside any claim — interest, a recovered overpayment
Check Amt	*Claims Pd* plus *PLB Adj*: what was actually deposited
PLB Rsn	Why the payer made the *PLB Adj*
Diff.	*Claims Pd* less what the lines below add up to. **It should be zero before you post**

24.2 The claims

Below, each claim on the check is a bold row, and its procedure lines are underneath. F9 opens every claim, F10 closes them.

Column	What it holds
ClaimKey / Bill-Code	The claim, and the procedure code on each line
Account, *SrvDate*, *PatName*	The encounter, the date of service, the patient
Expected	What the claim expected the payer to pay
Paid	What the payer paid
Adj	The amount written off — the contractual adjustment
Deduct	The patient's deductible or coinsurance on this line
CoIns	The amount to bill to the complementary payer
Pending	What will still be owed after posting
AdjCodes	The payer's reason and remark codes, with their meaning
ICN	The payer's number for the claim

The dot on each row says, before you post, what posting will do:

Dot	Means
Green	Nothing will be left pending
Yellow	Paid, but something will still be owed
Red	Nothing paid — a denial
Black	Overpaid
Grey	The claim was not found in Mp10; this line cannot post

Open Claim (right-click) opens the highlighted claim to look at, and **Print Single Claim 835 EOB Notice** prints the payer's own text for that claim. **Search** (F8) finds a claim, an account or a patient on a long check.

25 Posting a remittance

25.1 Before you post

The import fills in what the payer paid, the deductible and coinsurance, the reason codes and the ICN. Most write-offs it leaves to you:

Contractual adjustments. The difference between your charge and what the payer allows (reason code CO-45, and others like it) has to be written off before the claim balances. To write off what is left on a line, right-click its *Adj* cell. On a claim row, the same writes off every line. To do the whole check at once, click the *Adj* column header: *Maximum Amount To Adjust* writes off every paid claim whose remaining balance is no more than the amount you give. Read the reason codes first — not everything unpaid should be written off.

Deductibles. Click the *Deduct* column header to choose what posting does with deductibles: **Add to patient responsiblity** moves them to the patient's balance (the default), **Ignore Deductibles** leaves them out.

Coinsurance for the complementary payer. When the claim has a complementary payer, what is in *Deduct* and *CoIns* is billed to it (see below).

The ICN. Check it is there; you can type it if it is missing.

Then watch **Diff.**: when it is zero, every cent of the check is accounted for.

25.2 Posting

1. Click **Save Remit** (Ctrl+S).
2. Click **Post Remit** in the *Actions* panel (or right-click → *Post Remittance*).

A progress bar runs, and Mp10 reports how many entries it posted: *Remittance has been posted to payments table.*

Posting a check, in one step that either completes or changes nothing:

- records a payment, an adjustment and the reason codes on every claim line;
- recalculates each claim's balances;
- sets each claim's **flag** — green, yellow, red or black — and adds a *Remittance Posting.* row to its status history;
- moves deductibles to the patient's balance, as you chose;
- creates or updates the **complementary claims** (below);
- records the payer's refusals in the **denial history** that the claim checks learn from.

Only lines that are not yet posted are posted. Posting the same check again posts nothing: *Already posted.* If Mp10 answers *Nothing posted*, the check number, payer or date on the left do not match the ones the remittance was saved with — correct them and post again.

25.3 There is no undo

A posted remittance cannot be unposted. Check *Diff.* and the dots before you post. A mistake found afterwards is corrected on the claim, with the payment window (see *Other ways to enter a payment*).

An Administrator can delete a remittance that has **not** been posted, from the right-click menu of the Remittances list (*Delete Remittance*): for instance, a check entered twice by hand.

25.4 Complementary insurance claims

When the patient has a second insurance that pays after the first (the claim's *Comp Ins*), posting the primary payer's remittance hands the rest over:

- The amounts in *Deduct* and *CoIns* are assigned to the complementary payer instead of the patient.
- **If there is no secondary claim yet, Mp10 creates it**: a claim for the complementary payer, billing exactly what was assigned, with the patient's secondary card. Send it like any other claim.
- If the secondary claim already exists, it is adjusted to the new amount.

Posting a check twice never creates a second secondary claim.

25.5 When a claim is not found

A grey dot means Mp10 could not tell which claim the payer was paying — the payer quoted a number Mp10 does not know. Those lines do not post; the rest of the check does, and the check stays in the list as partly pending.

To fix one:

- Type the right claim number (or the encounter number) in its *ClaimKey / Bill-Code* cell; the claim's lines load. Save and post again.
- **Re-Fetch Missing Claims** (right-click in the Remittances list) tries the match again from the original file — useful after a patient or claim has been corrected.
- **Change Insurance Info** moves the whole check to the right payer, when it was filed under the wrong one.
- **Re-Fetch From 835** throws away an unposted remittance and imports it again from its file.

26 Entering a remittance by hand

A paper EOB is entered in the same window, so that it is posted, reconciled and remembered exactly like an electronic one.

1. Click **Start New Remit**.
2. On the left, enter *Chk Num*, the payer (*Ins*), *Chk Date*, and *Claims Pd* — the total the EOB says was paid on claims. Add *PLB Adj* if the payer withheld or added money outside the claims.
3. Add the claims on the EOB:
   - press Insert (or right-click → *Add New Line*) and type the claim number in *ClaimKey / Bill-Code*: the claim's lines load; or
   - click **Fetch Claims Set** to load many claims at once. Its window has three sources, one per tab, and the choice at the bottom follows the tab you work on and the file you pick:
     - *Dates* (the default) — the unpaid claims of the ticked payers for a range of service or billing dates;
     - *EOB from Scanner* — pick the EOB's PDF or scanned image (TIFF, PNG, JPEG), or click *Acquire EOB From Scanner* to scan it, or click **AutoTasks review folder...** to pick one AutoTasks held back (see *Loading an EOB's amounts*). Mp10 reads the EOB, scanned pages through OCR, and shows a preview of its lines before anything is loaded. On a new remit the check number and date are filled in when the EOB shows them. If the layout is not recognised, only the claim numbers are read;
     - *From list of ClaimKeys* — type claim numbers (down-arrow past the last row opens a new one), paste them with Ctrl+V, or pick any other file (Excel, text, CSV, Word) on the EOB tab: its claim numbers land here and this tab comes forward.

     Run loads the claims of the payer in *Insurance Co* that have no insurance payment yet; the count of claims found and matched shows on the EOB tab.
4. On every line, **enter what the EOB says**. Mp10 fills *Paid* with the expected amount to save typing, so correct each one that differs. Enter *Adj*, *Deduct*, *CoIns*, and the *ICN* from the EOB.
5. Enter the reason codes in *AdjCodes*, one per line, as `[45]-` followed by the description. Codes entered this way count toward the denial history, like the ones that come in electronically.
6. When *Diff.* is zero, **Save Remit**, then **Post Remit**.

26.1 Loading an EOB's amounts

On the *EOB from Scanner* tab, picking a PDF or scanned image (or typing its path) reads it. A progress bar shows the page being read, and **Cancel** on it stops the read: nothing is loaded. The file name stays in the field but is not read again when you click elsewhere; pick the file again, press **Run** or open it from the review folder to read it. While an EOB is being read, **Run**, the file picker, the scanner button and the review folder button do nothing. Enter *Insurance Co* first; it decides which claim receives the lines when a claim has a complementary insurance.

The **EOB preview** window then lists every line the EOB prints, with the check number, date and payer on top, how many pages needed OCR, any warnings, and any remittance in Mp10 that already holds this check or this PDF. It opens at the size you last left it.

- **Load** (the first column) is the tick. Click **Tick all** or **Untick all**, or press the space bar or double-click on a line to tick or untick it.
- A line starts **unticked** when something is wrong with it. Its *Reasons* column says what, in words: the claim was not found, the code is not billed on the claim, the claim was moved to the complementary claim, the line was already paid by this payer on another check, or the paid amount is more than allowed, billed, the check total or the ceiling. Lines that need attention are red; lines moved to a complementary claim are blue. You may tick any of them if you are sure.
- **Load into remittance** loads the ticked lines. **Cancel** loads nothing.

Only the lines the EOB prints are loaded, denied ones included. A line the claim has but the EOB does not print is left out, so the window holds the same rows AutoTasks would write for the same EOB. Per line,

Paid, *Adj*, *Deduct*, *CoIns*, the reason codes and the *ICN* come from the EOB. The *ICN* is filled in only when no page of the EOB was read by OCR (a scanned number is shown in the preview but not stored, because a misread number would overwrite the claim's). A claim the EOB prints in two blocks is loaded as two groups of lines.

After loading, *Claims Pd* is the total of the loaded lines and the box under the EOB tab's buttons says how many claims and lines were loaded and how many left out (scroll the box when it holds more lines than it shows). If the EOB prints a check total that differs from *Check Amt*, the tab says so (*missing lines or PLB*); this is a warning, not a block, so add the *PLB Adj* or the missing lines. Nothing is saved until **Save Remit**, and nothing is posted until **Post Remit**.

Messages you may see instead of the preview:

- *This EOB is already in Remits: Check …, … – posted.* The check is posted. Nothing is loaded; open it from the Remittances list.
- *These claims are already paid by this payer on a posted check.* Most of the EOB's lines were paid before. Nothing is loaded.
- *Could not check Remits for this EOB; try again.* Mp10 could not read whether the EOB is already in Remits, so it loads nothing rather than risk entering it twice. Try again in a moment.
- *This EOB is made out to another practice.*
- *This EOB's layout was not recognised: only its claim numbers were read.* The claims load as in the paragraph above, with *Paid* set to the expected amount for you to correct.
- *Save or clear the current remittance first.* A second EOB cannot be loaded while the window holds changes that are not saved. The message comes before the EOB is read, whatever its layout, so no time is spent on OCR. Save Remit (or start a new one), then load the next EOB. A Save Remit that failed leaves the window unsaved too, so its lines are not lost. Once the window is saved, the next EOB starts a new remittance by itself (the saved one leaves the window, *Insurance Co* stays).
- *The EOB's file name is too long for the FileName column in Remits.* Rename the PDF and pick it again. Save Remit checks the same thing again, because the *Check #* is part of the name: if you lengthened the *Check #* after loading, Save refuses and saves nothing. Shorten it, or clear the window, rename the PDF and load it again.

If the check is already in Remits but **not posted** (an earlier import got part of it), Mp10 opens that remittance and Load adds only the claims it does not have, including claims the check holds under another payer that the window does not show. A line the check already holds under the primary claim is not loaded again under the complementary claim. *Claims Pd* is then the total of every line in the window. **Cancel** leaves the remittance as it was opened.

Save Remit records the EOB's file name (`835_<check>_<pdf name>`, the same name AutoTasks gives) on the rows it loaded from the EOB, so a later AutoTasks run sees the EOB as already imported. Every other row keeps its file name: remittances not loaded from an EOB, and the rows a check already had when an EOB was added to it. Such a check shows in the Remittances list as two file name groups, as when AutoTasks adds to a check. When the PDF is in the watched folder (`EOBIMAGESPATH`) or its `review` sub-folder, Save also moves it to the `history` sub-folder (creating it when it is missing), with the JSON and reason file `review` holds for it; a PDF picked from anywhere else (Downloads, the desktop) is never moved. The folder counts however you reached it: through a mapped drive letter or its network (`\\server\share`) path. If a file cannot be moved, Save says so; the remittance is saved all the same. If Save Remit could not write every line, the PDF stays where it is: run Save Remit again.

AutoTasks review folder... lists the EOBs waiting in `review` (newest first, with the date, the refusal, the check, the PDF and where the PDF is). Pick one and it is read and previewed as above.

27 Other ways to enter a payment

27.1 A payment on one claim

Post Payment in the Claims grid, or **Add New Payment to Claim** on the claim's **Pymnts** tab, opens the *Single Payment Dialog*. The claim must be saved first.

On the **New Payment** tab, enter the *Check #*, *Check Date* and *Deposit Date* (they are remembered for the next claim), then, on each procedure line:

Column	Enter
InsPaid	What the insurance paid
Substract	The insurance write-off for the line
PtePaid	What the patient paid
PSubstct	The patient write-off
CoIns	What to bill to the complementary payer
AdjReason	Why, from your list of adjustment reasons

The buttons above the grid fill columns for you: the deductible as paid, the expected as paid, the insurance balance as coinsurance. The cell buttons do the same line by line.

The **Simple Adjust** tab adds to or subtracts from the whole claim's insurance or patient balance in one entry, with a reason and a note. **Save simple adjustments separately**: when any Simple Adjust amount is filled in, the line payments on the New Payment tab are not saved in the same Save.

The **Previous** tab lists everything already posted to the claim.

When you save, the claim's balances and flag are recalculated, and a secondary claim is created if you assigned coinsurance and the claim has a complementary payer.

27.2 Patient payments

Payments from the patient are posted with **Pat Payment** — from the patients grid at the front desk (Part I), or from the Claims grid, which ties the payment to that claim and prints a receipt.

27.3 Adjusting balances in bulk

Actions → *Adjust Accts* (Supervisors and Administrators) writes off small remaining balances across many claims: by dates of service, a range of balances, the payers you tick, and a reason. It first prints the list of claims it would adjust, and asks you to confirm before it changes anything — **Create report only** is ticked to begin with, so a first run only shows you. Adjusted claims are marked paid.

27.4 Refunds

Mp10 has no refund function. An overpaid claim shows a black flag; ask your administrator how your practice records a refund.

28 Why post whole remittances

Typing a check's payments claim by claim in the payment window balances the claims, but it leaves out everything else the remittance knows. Posting the remittance itself — electronic, PDF, or a paper EOB entered in the Remittances window — gives you four things the payment window cannot:

The ICN, for corrections. The remittance carries the payer's number for every claim, and Mp10 puts it on the claim. A corrected or replacement claim must quote it, or the payer rejects it as a duplicate. Posted from a remittance, it is already there when you need it.

The denial history. Every refusal on a posted remittance is remembered — which payer, which codes, which reason — and the claim checks learn from it. The **Denied Before** tag warns the biller, before the claim goes out, that this payer often refuses these codes. **Refused For This Patient** warns, at the front desk when the service is ordered and again on the claim, that the payer already refused it for this patient. The dashboard's rejected claims list explains each remittance denial with its reason codes. **All of this reads the remittances only.** A denial typed into the payment window is invisible to it, and the next claim goes out with the same mistake. When a new version of Mp10 learns something new from the history, the administration module's **Update Data Dictionary** rebuilds it, with a progress bar, and skips the rebuild when there is nothing new to learn.

Reconciliation. The remittance must add up to the check: *Diff.* shows you, before you post, whether every cent is accounted for, and *PLB Adj* explains what the payer withheld. The payment window has no check total to compare against.

The flags and the secondary claims. Posting sets every claim's flag and status history in one step, and creates each secondary claim with exactly the amounts the primary payer assigned.

So: post every remittance whole, and use the payment window for what is not on a remittance.

29 Reports for payment posting

Report	Shows
Print Remittance (Remittances window)	The open remittance, claim by claim
X12-835 Remittances Report (right-click the Remittances list)	The payer's file itself; with several checks selected, totals by payer
Reports → Remittances Report	Remittances by check date, by payer or check by check, with paid, adjusted, deductible and retained totals
Reports → Payments + Adjustments	Payments and adjustments for a period — by payment date, deposit date or the date posted — grouped by payer, check, user or referring physician
Reports → Claims Status	Claims with an insurance balance, by flag, and how long each has waited
Reports → Aeging of Accounts	What is owed, by age

Every report can be printed, exported, or scheduled to be e-mailed automatically from its *Automate Report* tab (Supervisors and Administrators).

Quick reference

At the front desk

To	Do
Find a patient	Patients Table, type the name, or F8
Add a patient	Insert; **save before** adding insurance or encounters
Scan the ID	Demographics → scanner button; the blank name, sex and address fields fill from the card (yellow) — check them, then Save. OCR re-reads and overwrites
Take the HIPAA signature	Demographics → pen → Accept
Add a new insurance card	Insurance → New → tick **Primary** → Save. The old card stays as history
Scan the card	While adding or modifying: Scan; a blank member number and group fill from the card (yellow) — check them, then Save. OCR re-reads and overwrites
Check eligibility	Patients grid → Eligibility → Fetch New Eligibility record
Open an encounter	Encounters tab → New → date, type, referring doctor → Save
Order	Encounter → Orders → New Order → Save
Take the visit signature	Encounters tab → highlight → pen → Accept
Mark a no-show	Open the encounter → Inactive encounter → Save
Take a copay	Pat Payment → encounter → lines → Save → Print

In billing

To	Do
See what is unbilled	Unbilled encounters (or the dashboard line)
Create a claim	Unbilled list → New Claim. Confirm each line's code so it prices
Code	ICD10 tab: pick a suggestion, or type words in *Description*. First code = principal
Point lines at diagnoses	*Diag*: `1`, `1,2`… Recheck after reordering
Read a tag	Hover it; click for the full explanation
Send	Claims grid → select one payer's claims → Outbound 837
See a claim's history	F9 on the claim
Record a follow-up	F9 → Add New → flag, date, **note** → Save
Work rejections	Dashboard → rejected claims → double-click, correct, resubmit
Resubmit	F4 → Corrected / Replacement / Void; check the ICN (`F8…`)

Posting

To	Do
Find a check	Remittances → Show Only Posting Pending → Reload Remits
Write off contractuals	Right-click *Adj*, or the *Adj* header for the whole check
Post	*Diff.* = 0 → Save Remit → Post Remit
Fix a grey line	Type the claim number, or Re-Fetch Missing Claims
Enter a paper EOB	Start New Remit → claims → amounts and reason codes → Save → Post